False, indisputable

False, because Gie City lied/withheld information

False/inaccurate, disputable

Subjective - projecting their values onto Gie City's life

ACT Government | Canberra Health Services

City Mental Health Team

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TO: Dr Randhir Ramchander

DATE: 13/08/2021

FAX NUMBER: 0262418550

NO. OF PAGES (INCLUDING COVER PAGE):

DATE FAXED: 13/08/2021

FROM: City Mental Health Team

URGENT: YES/NO **CONFIDENTIAL**: YES

Dear Dr Randhir Ramchander,

RE: Giles Cullity DOB: 22/07/1993

Please see below details from my recent contact with this patient.

Diagnosis

Schizoaffective disorder – in partial remission

Psychotropic Medications

- Clozapine 300mg PO nocte (commenced 19/1/2017 in SA)
- Aripiprazole LAI 300mg IMI monthly (last 27/7/21, next 24/8/21; record of administration in SA going back to Feb 2020)
- Venlafaxine XR 150mg PO mane
- Metformin MR 500mg PO nocte with meals for 7 days, increasing to 1000mg if tolerated (commenced 13/8/21)

Background

28 y.o. single unemployed man on DSP with NDIS living with family in [redacted] recently relocated from Gungahlin. Born Scotland, from Adelaide (relocated to CBR with family in 2021). History of homelessness in Adelaide.

Established diagnosis of schizoaffective disorder (predominant depression) with history of admissions and treatment under the MH act (in SA?). Collateral suggests partial response to clozapine with chronic somatic (delusional) complaints,

First contact with SA MH services in late adolescence. First contact with ACT MH services in Jan 2021 – linked with Gungahlin MH and clinically managed as a voluntary client. Nil admission in ACT.

Noted forensic history – assaulted police 2010, attempted murder in 2013 – cut the neck of another homeless person in the context of altercation over ownership of a mattress.

Family history of BPAD (brother) and ASD (youngest brother)

MHA Status: Voluntary

Referral

Transfer of care from Gungahlin MH in July after Giles moved with family to [redacted].

Progress

- Last psychiatric review Dr Khaira (Gungahlin registrar) 9/6/21: stable mental state, residual chronic grandiosity, negative syndrome, ?comorbid OSA
- Attended CMH for depot on 27/7/21

Assessment

I met with Giles today for the first time at City MH.

He describes feeling reasonably stable in his mental state and similar in presentation cf: his last psychiatric review in June.

Giles recently relocated with his family to a property in [redacted] and is adjusting reasonably well to the change. He states he feels "energised" by the move. He reports he currently unemployed but stays busy working out, reading philosophy, surfing the internet and working on his software idea. He articulately described developing a hybrid file storage and web-browsing solution which intuitively arranges information in a tree-like file structure to ease of "research". Whilst this may perhaps be outside of his current skill set, the

idea appears full formed and not of delusional intensity on brief screening today.

Giles reiterated that he disagrees with his diagnosis of schizophrenia/schizoaffective disorder – believes he suffers from "cyclothymia" or milder BPAD. States he has never experienced perceptual disturbance and thinks that his "judgmental errors" have been misinterpreted as delusions. He wants to explore whether he could potentially wean and cease his clozapine at some point in the future – I stated that there would likely be some reluctance from a service perspective given the seriousness of his assault charges in the past and the proposed linkage of these events to his illness. His main issues with clozapine are weight gain and sedation.

I explored the possibility of a trial of metformin – Giles was enthusiastic about this though I recommended caution given his history of renal impairment secondary to ?overuse of bodybuilding supplements and/or muscle injury secondary to weight lifting. I suggested gradual up titration and cessation at the appearance of muscle soreness. He agreed to a repeat test of renal function to coincide with his next set of bloods.

Giles is aware that I will be leaving City MH and that he will be following up with a different doctor in ~3/12.

MSE

A+B	Young man, average stature, muscular build. Short brown hair, reasonable grooming. Easily engaged, open and forthcoming. Appropriate eye contact. Settled posture. Nil marked psychomotor changes or abnormal movement
M+A	Mood is "up and down" but impresses as euthymic, blunted affect
S	Somewhat monotonous
T_F	Nil marked FTD, normal stream
T_C	Range of appropriate themes, described in some depth his proposed software project which appeared reasonably logical though perhaps outside of his skill set – it has been suggested that this is a residual grandiose delusional construct but did not appear of that intensity on cross-section today. Denies thoughts of self-harm or harm to others
P	Denies disturbance, nil evidence of responding
C	Alert and orientated to TPP
I+J	Limited insight but accepting of current treatment. Judgement and capacity impacted marginally by lack of insight but open to direction of health professionals

Risk Profile

Suicide / Self-Harm Risk Factors

Static	Dynamic
Male 25 – 50, >70 years Nil known history of self-harm	Denies current thoughts of self-harm Relationship problems / breakdown Social Isolation Unemployed

Violence / Aggression Risk Factors

Static	Dynamic
Male sex History of assault of police (2010) and attempted murder (2013) whilst unwell	Currently settled, nil evidence of psychosis on review

Impression

Schizoaffective disorder – perhaps some residual over-valued grandiose ideas related to his software project but not acutely unwell on review. Negative symptoms of functional withdrawal, affective blunting present.

Does not appear depressed or elevated on cross-section.

Recommendation & Plan

1. Continue clozapine, aripiprazole, venlafaxine
2. Commence trial metformin MR 500mg PO nocte (with dinner) for 7 days, increasing to 1000mg PO nocte if tolerating – script 500mg MR tab [120] 5x repeats
3. History of renal impairment and elevated CK ?secondary to inappropriate creatine use – have included UEC on next set of bloods to monitor. Patient provided with pathology form
4. Suggest continuing 3-monthly psychiatric clozapine reviews

If you have any questions or require further information regarding the above, please feel free to contact me via reception at City Mental Health on 02 5124 1795.

Kind regards,



Dr Perrin Kwek

Psychiatric Registrar

City Mental Health

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